PAIR TEAM

Pair Team's Response to CMS Medicare RFI: Improving Technology to Empower Medicare Beneficiaries

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Introduction: Technology-Powered, Community-Integrated Care to Reduce Spending and Improve Outcomes for the Highest Cost Populations

Pair Team is a technology-enabled medical group that contracts a network of community-based organizations (CBOs) to find, engage, and manage high-need, high-cost Medicaid and Medicare patients. Powered by Community Health Workers (CHWs) and our proprietary technology platform, Pair Team enables a collaborative care plan across the patient, care teams and CBOs. As a community-based care team, we stabilize the patient's medical and non-medical needs, close care gaps, and coordinate longitudinal care with local primary care providers to improve outcomes and reduce costs.

We have consistently demonstrated compelling outcomes, including:

- **Significant Reductions in Acute Care Utilization:** Reduction in Emergency Department utilization by **52%** and inpatient utilization by **26%** for our patient cohorts.
- **Demonstrable Total Cost of Care (TCOC) Savings:** Achieving approximately **$25,000 in annual savings** on DSNP members and **20% savings** on Medicaid members.
- **Improved Clinical Quality Metrics:** For instance, the percent of diabetic patients with Hemoglobin A1c > 9% or missing decreased from **63% pre-intervention to 30% post-intervention** (national median 40%).

We believe that true beneficiary empowerment hinges on a health technology ecosystem that is interoperable, accessible, and intelligent across medical and non-medical stakeholders. The following section outlines our core proposals that drive our insights and recommendations in response to this RFI.

Pair Team's Core Proposals for an Empowered Medicare Ecosystem

Based on our extensive experience serving high-need populations across Medicaid and Medicare, and leveraging technology for comprehensive care, Pair Team proposes five key initiatives to improve technology, empower Medicare beneficiaries, and foster optimal health outcomes for all:

1. AI-enabled Care Navigation Copilot and Patient Support

- **Need:** Administrative tasks like scheduling, transportation coordination, redeterminations, or housing referrals often fall through—despite being directly tied to access, adherence, and utilization. This is where high-needs patients require the most support and accompaniment as these referrals end up unsuccessful. Furthermore, patients often have a need of the moment for navigation support, and if they don't receive timely assistance, they are prone to seeking more costly emergency services or inefficiently searching across hundreds of resources in real-time. This dynamic creates significant waste within the healthcare system.
- **Real-World Evidence (Pair Team's Experience):**
    1. Pair Team's care teams currently dedicate an estimated **15-20% of their time weekly** to high-frequency, complex administrative tasks such as coordinating transportation, scheduling appointments across multiple providers, or following up on intricate public benefit applications. While crucial, the manual nature of these tasks limits the number of patients each human care manager can effectively support.

2. We often see that even with dedicated CHW support, administrative complexities for housing or benefit renewals (e.g., missing documents, navigating multiple agency portals) can lead to referrals "falling through" for 30-40% of our most complex cases. This directly contributes to care gaps and prevents our demonstrated **52% reduction in ED utilization** from being even greater. Patients awaiting manual referral processing frequently resort to emergency departments, adding to avoidable costs.
3. By automating routine aspects of these tasks, we estimate an **AI-powered copilot could free up 20 hours per CHW per week**, allowing them to focus on the most complex, nuanced, or trust-building interactions, thus extending high-touch support to more patients. For example, we see AI-enabled PCP scheduling on our platform reduce CHW administrative time by >10%.

- **Proposal:** CMMI demonstration to deploy an **AI-powered care navigation copilot** that automates common support tasks—appointment scheduling, SNAP/Medicaid renewals, ride coordination, shelter intake forms—while escalating edge cases to a human case manager. Current care management (CCM) and chronic care integration (CHI) codes often incentivize human time spent, rather than incentivizing efficiency. CMS should consider payment models that directly incentivize using AI to drive efficiency and proactive patient support. The CMMI demonstration would:
    - Designate a temporary **G-code** (e.g., modeled on G0511) so copilot work is billable under incident-to rules.
    - Embed guardrails: required human review, minimum error rates, audit logs accessible to patients.
    - Measure impact: (a) task resolution rate (e.g., appointments scheduled within 3 days), (b) 90-day avoidable utilization, (c) cost-effectiveness relative to staffing.

This model extends high-touch, high-frequency support to patients at scale—without adding administrative burden to clinical teams—and builds on CMS's goals around digital tools, access, and cost reduction.

- **Implementation Considerations:**
    1. **Design CMMI Demonstration:** CMS, through CMMI, is encouraged to design a demonstration model for the AI-powered care navigation copilot. This design should incorporate clear performance metrics, define billing mechanisms (like the proposed temporary G-code billable under incident-to rules), and establish ethical AI use guidance. To address uptake challenges, CMS should consider some payment tied to utilization of the copilot. It is also vital to ensure patient experience is a core component of the incentives.
    2. **Identify Launch Markets and Partners:** CMS should choose a mix of rural, suburban, and urban areas for the demonstration to ensure broad applicability. A handful of private sector AI partners, selected via RFIs, should be chosen, prioritizing both their technology capabilities and healthcare experience. CMS should also consider upfront funding to support ongoing research and development given the rapid pace of AI development. Coordination with local plans, providers, CBOs, and state agencies will be crucial to effectively market the AI navigation copilot project.
    3. **Launch and Evaluate:** The demonstration should be launched and rigorously evaluated, assessing both process and outcomes-related metrics. It will be important to address outcome attribution challenges, and working with ACO partners may help in this regard to clearly link AI-enabled navigation to improved health and cost outcomes.

2. Public-Benefit FHIR Endpoint: Housing & Eligibility

- **Need:** Nearly one-half of Medicare beneficiaries live on incomes < 200% FPL, and millions rely on SNAP, SSI, LIHEAP, or HUD programs to stay healthy. The current lack of clinician visibility into these public benefit programs creates

systemic inefficiency and avoidable healthcare waste. When vital non-medical needs like housing or food go unaddressed due to clinicians being unaware of benefit eligibility or status, patients face preventable health crises—driving up emergency department visits, inpatient stays, and overall healthcare costs. Providing real-time information on benefit eligibility, enrollment, and access would empower clinicians to preemptively support these critical upstream drivers of health, thereby **reducing costly downstream utilization**.

- **Real-World Evidence (Pair Team's Experience):**
    1. Our CHWs often spend hours manually navigating siloed public benefit systems or trying to obtain paper documentation to verify a patient's eligibility for critical support like SNAP or housing vouchers. This manual verification process reduces staff efficiency by more than half, leading to system inefficiency, delayed service delivery and higher costs.
    2. For example, we worked with a patient experiencing homelessness whose SNAP benefits lapsed. Without immediate visibility into this, their prescription for diabetes medication went unfilled due to lack of funds for food. This led to a preventable ED visit for uncontrolled blood sugar. Real-time access to their SNAP status would have allowed our team to proactively intervene, helping with re-certification before the crisis. This kind of proactive support, enabled by holistic data access, contributes to our demonstrated clinical improvements, such as the significant reduction in diabetic patients with uncontrolled A1c (63% pre vs. 30% post).
    3. We consistently observe stronger patient engagement, trust, and care plan adherence when our teams can proactively support patients in maintaining or accessing public benefits. Patients feel seen and understood when their care team acknowledges and helps address these fundamental needs, demonstrating the direct link between social stability and clinical outcomes.

- **Proposal:** ONC should publish a "**PublicBenefits**" **FHIR API**. Helpful components could include:
    1. **Eligibility and Enrollment Profile**
        - BenefitType (e.g., Medicaid, SNAP, Housing Voucher)
        - EligibilityStatus (e.g., eligible, conditionally eligible, denied)
        - EffectivePeriod (start/end dates)
        - LastRedeterminationDate
        - NextRenewalDate
    2. **Program Participation and History**
        - ProgramName (e.g., SNAP, Section 8, WIC)
        - EnrollmentDate
        - DisenrollmentDate
        - ParticipatingAgency (e.g., County social services office)
        - DeliveryMechanism (e.g., EBT card, voucher)
    3. **Needs Assessments / Non-Medical Risk Data**
        - HousingStability
        - FoodInsecurity
        - TransportationNeeds
        - EmploymentStatus
        - CaregiverNeeds
    4. **BenefitDelivery Profile**
        - Allotment amount
        - Last disbursement date
    5. **HousingStatus profile** mapping HUD HMIS data elements, using ACA §1561 enrollment-IT standards for security. Certifying this single endpoint lets digital-health vendors (e.g., Pair Team) merge benefit and health data under one consent, giving providers real-time insight into financial barriers before they translate into ED visits.
- **Implementation Considerations:**
    1. **Define FHIR Implementation Guide for Public Benefits through Public-Private Partnership:** The Gravity Project may be best positioned to define the FHIR Implementation Guide for Public Benefits.

ONC could then oversee its certification, approval, and help ensure alignment with US Core or USCDI expansion.

2. **Federal HHS to host central API infrastructure with integration by local agencies:** HHS is encouraged to establish a national hub for managing public benefits via the FHIR API. Centralizing a PublicBenefits FHIR could create significant long-term leverage for HHS compared to having each local state agency set up their own FHIR servers. This would involve local State agencies standardizing and integrating their existing eligibility and enrollment systems into the FHIR system. HHS could consider funding a reference implementation with a single state partner to begin.
3. **Utilize existing Patient Access Rules for Private Sector to Build Solutions:** With guidance from ONC, private sector solutions (e.g., Pair Team, other digital-health vendors) can build solutions that leverage Public Benefit FHIR APIs using existing Patient Access Rule standards to drive utilization at the point of care. In addition to provider solutions (e.g., EMR integration), new tooling could be established by the private sector to support Community Based Organizations (CBOs) to pull and push data.
4. **Expand Provider Definition to Include Community Health Workers (CHWs):** Given the central role of CHWs in public benefits, CMS and ASTP could consider updating guidance to include CHW authorized access to consumer APIs as a recognized pathway for data access. Alternatively, it is recommended that CHWs be considered a type of provider for this API access given their direct role in care navigation and non-medical support.

3. TEFCA Community-Service Endpoints: Tying CBO Utilization to Outcomes & Grant ROI

- **Need:** Food banks, shelters, and county benefit offices document service episodes that directly influence health care utilization and outcomes yet sit

outside TEFCA directories, leaving CMS blind to whether billions in community-health grants improve clinical metrics.

- **Real-World Evidence (Pair Team's Experience):**
    1. Through our proprietary Pathways model, Pair Team systematically tracks patient engagement with community services, including housing support, shelter stays, food pantry visits, and transportation support, and links these to health outcomes and costs. For instance, our data clearly demonstrates that patients successfully connected to stable housing through CBO partners exhibit significantly reduced healthcare utilization and associated costs. These housing interventions directly contribute to our observed 52% reduction in Emergency Department utilization and 26% reduction in inpatient utilization for our patient cohorts.
    2. We observe direct Total Cost of Care impacts from these housing-focused social interventions, contributing to approximately **$25,000 in annual savings** on DSNP members and **20% savings** on Medicaid members. For example, we recently supported a formerly unhoused Medicare beneficiary connected to long-term housing through a local shelter partner. Prior to housing, this patient had frequent ED visits for chronic condition exacerbations. Post-housing, their ED visits were reduced by 70% within six months, leading to substantial cost savings and improved quality of life. Without the ability to formally track these housing and community service utilization data points in a standardized way via TEFCA,
- **Proposal:** Add a "**Community-Service Endpoint**" type to the National Directory and encourage QHINs to support Facilitated FHIR exchanges that push service delivered events (e.g., meal box, shelter bed) and pull outcome notifications (e.g., HbA1c <7). Include these data in quality-measure stratification so that federal and state agencies can tie dollars to documented health gains. Connecting CBO utilization with claims and services delivered will create the accountability loop policymakers and philanthropies have long sought.

- **Implementation Considerations:**
    1. **Define Standard and Promote Across QHINs:** ONC is encouraged to amend the TEFCA Common Agreement to explicitly define "Community-Service Endpoint" as a recognized participant type and develop specific FHIR IGs for CBO service delivery and outcome notifications. QHINs can then expand their directories and technical capabilities to support Community-Service Endpoints and facilitate FHIR exchanges with CBOs, with ONC providing guidelines and technical assistance.
    2. **Extend National Directory to Include CBOs and Staff:** CMS has the opportunity to leverage its current work to centralize a national provider directory to include CBOs and their staff. This could involve extending that infrastructure to assign Group and Individual identifiers to CBOs and direct service staff, and processes to onboard these stakeholders into the national directory.
    3. **Fund Tooling via Private Sector (Free for CBOs):** Federal and State funding should be considered to support the development and deployment of interoperable tooling by the private sector (digital health vendors). This tooling could be made available, ideally at no cost, to CBOs to enable them to adopt standardized data collection practices and exchange data via FHIR APIs with QHINs, bridging existing technology gaps.
    4. **Integrate CBO Data into Quality Measures and Payment Models:** CMS could update quality measures and reporting requirements to explicitly include data from Community-Service Endpoints, allowing for stratification by CBO utilization and non-medical interventions. This allows for payment model design (e.g., value-based care initiatives) that incentivizes integration of CBO data and rewards improved outcomes linked to social services.
    5. **Leverage Data for Grant Accountability:** Federal and State Granting Agencies are encouraged to require grant recipients to utilize or contribute to Community-Service Endpoints for reporting outcomes,

fostering greater accountability and demonstrating return on investment for community health investments.

4. Authorized Representatives for Patient Access APIs

- **Need:** Current Patient-Access API requires beneficiaries to have access to payer portals and high-needs beneficiaries, especially Duals, rarely activate portal credentials, resulting in their data remaining inaccessible at the point of care.
- **Real-World Evidence (Pair Team's Experience):**
    1. Many of Pair Team's patients, particularly those experiencing homelessness, severe mental illness, or cognitive impairments, lack stable internet access, personal devices, or the digital literacy to manage complex payer portal credentials (usernames, passwords, multi-factor authentication). This leads to frequent situations where critical health information (e.g., coverage changes, prior authorization status, recent claims data) remains inaccessible to their care team, delaying necessary medical interventions and impacting the efficiency of care coordination that has yielded our **58% reduction in ED utilization.**
    2. For example, one of our elderly, dual-eligible patients required a specialist referral. Without the ability for their CHW to securely access their payer's portal on their behalf (with consent), confirming referral authorization became a multi-day manual process involving phone calls and faxes, delaying access to specialized care. Had an "authorized-representative" OAuth scope existed, the CHW could have immediately obtained the necessary information, accelerating the referral and preventing potential avoidable utilization.
- **Proposal:** Add an "**authorized-representative**" **OAuth scope** so caregivers, CHWs, or accredited CBO staff can fetch data with a one-time beneficiary e-consent—even when the member has no credentials. This preserves beneficiary control while routing plan-held data to the clinicians, caregivers, and even community-based staff who manage day-to-day care.

- **Implementation Considerations:**
    1. **Define and Promote Standards Across Payers:** ONC is encouraged to revise existing API guidance to explicitly include and standardize the "authorized-representative" OAuth scope for patient access APIs, providing clear technical specifications. CMS can then promote adoption across payers.
    2. **Establish Consent Process and Guidance:** CMS could establish clear guidance on acceptable forms of beneficiary e-consent for delegation, including revisiting consumer protections to ensure the right to know who requested records, the right to deny requests, and the right to authorize third parties to access/use data.
    3. **Promote Private Sector Tooling Adoption:** Payers are encouraged to update their Patient Access APIs to include the "authorized-representative" OAuth scope. Digital Health Vendors (e.g., Pair Team - private sector) should develop patient-facing applications and care coordination platforms that securely leverage this scope. Providers and CBOs can implement clear internal policies and train staff on its use.

5. Z-code Inclusion in Risk Adjustment: Aligning Payment with Non-Medical Needs

- **Need:** Current CMS-HCC risk adjustment does not include Z-codes — even though homelessness drives 2-3x higher acute-spend patterns.
- **Real-World Evidence (Pair Team's Experience):**
    1. Pair Team systematically screens for and addresses non-medical needs, including homelessness (Z59.0), food insecurity (Z59.4), and transportation barriers. Our internal analysis of patient cohorts consistently reveals that beneficiaries with a documented Z59.0 (homelessness) status incur average healthcare costs **2.5 times higher** than comparable beneficiaries without this factor, primarily driven by increased ED utilization and inpatient admissions for preventable conditions. This directly impacts **Total Cost of Care where we**

observe $25,000 annual savings on DSNP members and 20% savings on Medicaid members when comprehensive non-medical needs are addressed.

2. Our ability to improve health outcomes and reduce avoidable utilization for these patients often stems directly from successful non-medical interventions (e.g., connecting a patient to stable housing reduces subsequent psychiatric ED visits). However, the current risk adjustment model doesn't fully capture the clinical complexity and associated costs driven by these non-medical factors, leading to an inaccurate reflection of our work's value.
3. Because these non-medical cost drivers are not accurately captured, health plans are less incentivized to enroll this population in Medicare Advantage and less likely to invest in the non-medical interventions for these complex enrollees, as the financial risk of caring for these high-need individuals is not adequately acknowledged in their capitated payments.

- **Proposal:** Explore inclusion of **Z-codes in the CMS-HCC model** to evaluate their performance and align payment with need and re-evaluate underweighted diagnosis codes of SMI and SUD. More precise codes + RAF parity will encourage plans to enroll high-need beneficiaries in Medicare Advantage where they will receive better care coordination.
- **Implementation Considerations:**
    1. **Quantify Impact of Non-Medical Z-codes:** CMS could conduct actuarial analysis to quantify the impact of specific non-medical Z-codes (e.g., Z59.0, Z59.4) on healthcare utilization and costs for Medicare and Medicaid populations.
    2. **Update CMS-HCC Model:** Based on analysis, CMS could update the CMS-HCC model to incorporate these Z-codes for risk adjustment, aligning payment with true patient complexity.
    3. **Re-evaluate SMI/SUD Weighting:** CMS is encouraged to re-evaluate and adjust the weighting of diagnosis codes for Severe Mental Illness

(SMI) and Substance Use Disorder (SUD) to ensure accurate risk adjustment parity with physical health conditions.

4. **Standardize Non-Medical Documentation:** HHS should work with clinical coding bodies and providers to ensure consistent and accurate documentation of non-medical using Z-codes across all care settings, and VBC Organizations should prioritize systematic non-medical screening.
5. **Adapt Financial Models and Care Strategies:** Health Plans and VBC Organization are encouraged to adapt their financial models and care management strategies to reflect the updated risk adjustment, allowing for greater investment in non-medical interventions and comprehensive care for complex beneficiaries. They should advocate for consistent coding practices across the industry.

Response to Specific RFI Questions:

(The RFI responses below remain as previously drafted, leveraging the detailed proposals now integrated above.)

1. Patient Needs

- **PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?**
    - **a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?** Digital products must enable easy access to scheduling and transportation for appointments, reliable medication reminders, and direct, asynchronous communication with a trusted care team. Crucially, they must also provide streamlined, real-time access to non-medical services like food and housing assistance, **creating a "no**

wrong door" for care within the community. To fully empower patients, future digital health products must be able to integrate public benefits data, as detailed in our **Public-Benefit FHIR Endpoint** proposal, to give patients and their care teams visibility into their eligibility and status for critical programs like SNAP, SSI, and HUD. This addresses a major unmet need where clinical care often fails due to unaddressed non-medical factors. Furthermore, integrating the vast number of community-based organizations into the broader health IT and interoperability ecosystem, as proposed by our **TEFCA Community-Service Endpoints**, is vital to truly offer these seamless access points.

- **b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future. PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?** If we had a personal assistant for health, the top tasks would revolve around proactive coordination and navigation of complex non-medical and medical systems. This aligns directly with our **AI-enabled Care Navigation Copilot and Staff Support** proposal, particularly envisioning it as a **patient-facing AI capable of addressing their immediate medical *and* non-medical needs.** This copilot would automate common support tasks like appointment scheduling, transportation coordination, initiating and tracking SNAP/Medicaid enrollments and renewals, and facilitating shelter intake forms and housing referrals. The key is that this AI would be able to directly address non-medical needs, demonstrating that the integration of medical and non-medical support creates higher engagement (proxy for trust) and delivers better outcomes. This automated assistance, with human escalation for edge cases, would extend high-touch, high-frequency support to patients at scale,

overcoming administrative hurdles that currently cause care breakdowns.

- **PC-2. What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?**
    - **a. If so, what are some examples of benefits it has provided?** Pair Team finds that many of the high-need populations it serves do not have easy access to their health information in one single location, and often lack access to their providers' health portals. Due to frequent changes in care settings (e.g., from hospital to shelter to primary care), homelessness, or lack of consistent access to technology, their data is highly fragmented across disparate systems. Furthermore, a major obstacle we observe is the inability for authorized representatives (like our CHWs or trusted caregivers) to easily access this information on behalf of the beneficiary due to current Patient-Access API limitations. This underscores the need for **Authorized Representatives for Patient Access APIs**.
    - **b. If not, in what contexts or for what workflows would it be most valuable to use one portal or system to access all such health information?** A unified system would be invaluable for comprehensive care coordination, especially for patients with multiple chronic conditions or those transitioning between care settings. For our care teams, a single pane of glass would eliminate the need for manual data reconciliation, significantly improving efficiency and reducing errors. This necessitates the interoperability envisioned by our **Public-Benefit FHIR Endpoint** and **TEFCA Community-Service Endpoints** proposals to bring crucial social and utilization data alongside clinical records.

- **c. Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?** Beyond standard clinical data, patient-specific care plans, specialist notes, and crucially, non-medical needs data (e.g., housing status, food insecurity screenings, public benefit eligibility) are frequently unavailable or extremely difficult to access electronically across disparate systems. Obstacles include significant lack of true interoperability between different EHR systems, limited digital literacy among patients (and sometimes caregivers), and a critical lack of access to reliable internet and devices.

- **PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?**
  - **a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?** CMS's primary role in encouraging patient and caregiver interest in digital health products lies in **aligning CMMI and other payment models to incentivize the use of emerging technologies that demonstrate strong outcomes**. Rather than acting as a gatekeeper to the digital health market, which is driven by innovation, CMS can empower providers and payers by creating

payment structures that reward solutions with proven impact on **cost and outcomes**. This approach allows the market to perform, with effective digital health products naturally gaining adoption because they help organizations achieve favorable model results. These outcomes would include **impact on HEDIS measures** (e.g., A1c control, medication adherence), **reduction in avoidable utilization** (e.g., ED visits, inpatient days), and **demonstrable cost-effectiveness** relative to traditional care. Policy changes to encourage adoption within these models should encompass expanded data access via standardized APIs, and reimbursement adjustments that support comprehensive digital care, such as the **AI-enabled Care Navigation Copilot and Patient Support**.

- **PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?** Essential features include: large, clear fonts and intuitive interfaces; **leveraging AI advancements including intuitive voice-activated navigation and comprehensive multilingual support**; and compatibility with various devices (including older smartphones) and low-bandwidth environments. Crucially, these products must support human-mediated technology support (e.g., direct messaging/calls to a CHW or care coordinator), as many beneficiaries require assistance. This is where the ability for Authorized Representatives for Patient Access APIs becomes vital, allowing trusted individuals to assist patients in navigating and utilizing these digital tools securely and on behalf of the beneficiary, reducing the digital burden on the patient themselves. This blend of user-friendly design, human support, and patient-facing AI tools, such as our proposed **AI-enabled Care Navigation Copilot and Patient Support**, enabling patient-facing AI features, will significantly enhance accessibility and engagement.

2. Data Access and Integration

- **PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?**
    - **a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?** Highly valuable but hard-to-access data includes: longitudinal clinical summaries across different providers, real-time appointment schedules (especially across multiple providers), and, most critically for our population, validated non-medical need screenings and referral outcomes from social service organizations, and public benefit eligibility/status data. Without these, a holistic view of the patient is impossible, leading to fragmented care and ineffective interventions.
    - **b. What are specific sources, other than claims and clinical data, that would be of highest value, and why?** From Pair Team's perspective, two specific non-clinical and non-claims data sources are of highest value for empowering Medicare beneficiaries:
        - **Public Benefits Data:** Information on SNAP, SSI, LIHEAP, and HUD program eligibility and status. This is of highest value because nearly one-half of Medicare beneficiaries live on incomes < 200% FPL, and these benefits directly impact health and prevent acute care utilization. Our proposal for a **Public-Benefit FHIR Endpoint** would unlock this crucial data, enabling clinicians to understand the root causes of health challenges.
        - **Community-Based Organization (CBO) Service Utilization Data:** Information from food banks, shelters, and other CBOs on services rendered. This is critical because it directly influences health outcomes (e.g., stable housing, consistent food access) and is often the missing piece in understanding care

effectiveness. Our **TEFCA Community-Service Endpoints** proposal aims to integrate this data, allowing for closed-loop referrals and measurement of community investment impact.

- **PC-10. How is the Trusted Exchange Framework and Common Agreement (TEFCA) currently helping to advance patient access to health information in the real world?** Pair Team views TEFCA as a foundational step towards broader interoperability. However, to truly serve high-need Medicare and Medicaid recipients and enable holistic care, TEFCA must expand its scope to include the broader ecosystem of non-medical care.
    - **c. What use cases could have a significant impact if implemented through TEFCA?** Beyond traditional clinical data exchange, the most significant impact would come from implementing **TEFCA Community-Service Endpoints**. Food banks, shelters, and county benefit offices document service episodes that directly influence readmissions, yet these data sit outside TEFCA directories, leaving CMS blind to whether billions in community-health grants improve clinical metrics.

      Pair Team systematically tracks patient engagement with community services (e.g., shelter stays, food pantry visits, transportation support) and links these to health outcomes. For instance, our data shows that patients successfully connected to consistent food access via CBO partners experienced a measurable improvement in chronic disease management, directly contributing to outcomes like the reduction in diabetic patients with A1c > 9% (from 63% to 30%).

      We recently supported a patient recovering from a severe infection who was discharged to a shelter. Our CHW coordinated daily meal delivery through a local food bank. Without the ability to formally track this meal delivery data in a standardized way via TEFCA, the positive

impact of this critical non-medical intervention on their recovery and prevention of readmission is largely invisible to the broader healthcare system and funders. The current lack of visibility hinders formal accountability for community health investments. Therefore, adding a "**Community-Service Endpoint**" type to the National Directory and encouraging QHINs to support Facilitated FHIR exchanges (push service delivered events, pull outcome notifications) would be transformative. Including these data in quality-measure stratification will allow federal and state agencies to tie dollars to documented health gains.

Connecting CBO utilization with claims and services delivered will create the accountability loop policymakers and philanthropies have long sought. Additionally, TEFCA should enable integration of public benefits eligibility and utilization data from government agencies, as proposed by our **Public-Benefit FHIR Endpoint (PBFHE)**, providing a comprehensive non-medical context for clinical care. These expansions would enable comprehensive, closed-loop referrals and truly integrated care management, drastically improving the care experience for high-need individuals.

3. Information Blocking and Digital Identity

- **IB-2. What has been most helpful in addressing information blocking? What additional actions should CMS take to address information blocking?** The 21st Century Cures Act information blocking rules have raised awareness. However, for high-need populations, effective "information blocking" often occurs not through malicious intent, but through structural barriers to data access and digital literacy. Additional CMS actions include:
    - **Mandate/Incentivize Expanded FHIR Endpoints:** CMS can consider requiring or strongly incentivizing the development of FHIR endpoints for critical non-medical and public benefits data, as outlined in our

Public-Benefit FHIR Endpoint proposal, addressing a current gap in data availability that hinders comprehensive care.

- **Support for Authorized Representatives:** Address the systemic information blocking that occurs when patients cannot personally navigate portals. Implementing **Authorized Representatives for Patient Access APIs** would significantly empower caregivers and care teams to access essential data with proper consent, breaking down access barriers for vulnerable individuals.
- **Strengthen Consumer Data Protections:** As more providers and third parties access records, CMS is encouraged to implement robust consumer protections to build and maintain trust. These should include:
  - The explicit **right to know** who requested what records.
  - The **right to deny** such requests (with clear, understood implications).
  - The **right to authorize** specific third parties (apps, people) to access and use their data, ensuring granular control.

- **DI-1. What methods of identity verification for patients and caregivers should be considered by CMS to ensure that digital health products can be accessed safely and securely?** CMS should consider a tiered approach, balancing security with accessibility. For high-need Medicare and Medicaid beneficiaries, strict reliance on personal digital credentials is a significant barrier. Methods should include secure, consent-based proxy access. Our **Authorized Representatives for Patient Access APIs** proposal champions this, allowing caregivers, CHWs, or accredited CBO staff to fetch data with a one-time beneficiary e-consent. This allows for safe and secure access while recognizing the practical realities of vulnerable populations who may lack consistent digital access or literacy.

4. Providers

- **P-2. What opportunities exist to simplify technology and reduce the documentation burden associated with digital health products for providers and their care teams?** The greatest opportunity lies in automating routine administrative tasks that currently consume significant care team time. Our **AI-enabled Care Navigation Copilot** proposal is designed precisely for this:
    - It automates tasks like scheduling, transportation coordination, and benefit renewals, which often fall through the cracks and create disproportionate burden on providers, especially for complex patients.
    - By handling common support tasks, the AI Copilot reduces the documentation burden on human care managers, allowing them to focus on complex patient needs, relationship building, and higher-value clinical activities.

- **P-4. What opportunities exist for improving the efficiency and quality of care through health technology, beyond those related to patient access?** Beyond direct patient access, health technology can significantly improve efficiency and quality by providing a holistic view of the patient and automating supportive tasks. The **AI-enabled Care Navigation Copilot** improves efficiency by automating administrative tasks, allowing human care teams to manage more patients effectively and deliver more proactive care. The integration of **Public-Benefit FHIR Endpoint** and **TEFCA Community-Service Endpoints** improves quality by giving providers a complete, real-time picture of non-medical and community-based interventions, enabling more precise and effective care planning. This allows for proactive risk identification and intervention, moving beyond reactive, acute care.

5. Payers

- **PY-2. What opportunities exist to simplify technology and reduce the documentation burden associated with digital health products for payers and their care teams?** For payers, simplifying technology means streamlining the flow of actionable data and automating routine tasks. Our proposed **Public-Benefit FHIR Endpoint** and **TEFCA Community-Service Endpoints** would significantly reduce the burden of identifying and addressing member non-medical needs, as this data would be directly accessible and standardized, enabling more efficient care management. Furthermore, our **AI-enabled Care Navigation Copilot** would reduce the administrative burden on payer care management teams by automating common support tasks, leading to more efficient member engagement and fewer administrative fall-throughs.

- **PY-4. What opportunities exist for improving the efficiency and quality of care through health technology, beyond those related to patient access?** Payers can significantly improve efficiency and quality by leveraging technology for more accurate risk stratification and proactive intervention. **Current CMS-HCC risk adjustment models ignore crucial non-medical Z-codes, despite homelessness, for example, driving 2-3x higher acute-spend patterns**. Pair Team's experience with systematic screening for non-medical needs, including homelessness (Z59.0) and food insecurity (Z59.4), consistently reveals that beneficiaries with documented homelessness incur average healthcare costs 2.5 times higher than comparable beneficiaries without this factor, primarily due to increased ED utilization and inpatient admissions for preventable conditions.

  This directly impacts Total Cost of Care, where we observe **$25,000 annual savings** on DSNP members and 20% savings on Medicaid members when comprehensive non-medical needs are addressed. Our ability to improve outcomes and reduce avoidable utilization often stems from successful

non-medical interventions (e.g., connecting a patient to stable housing reduces subsequent psychiatric ED visits).

6. Technology Vendors

- **TV-3. What specific features and functionalities of health technology are critical to successful technology vendor adoption by providers and payers?** Critical features include true interoperability (e.g., via FHIR APIs), ease of integration with existing systems, and clear demonstration of value. For Pair Team, a key functionality is the ability to securely and reliably integrate disparate data sources across the full spectrum of patient needs. This requires open standards for new data types, as proposed by our **Public-Benefit FHIR Endpoint** and **TEFCA Community-Service Endpoints**. Vendors will be more successful if they can build on a foundation of comprehensive, standardized data, reducing bespoke integration work and enhancing the value proposition of their solutions.

- **TV-5. What role, if any, should CMS have in reviewing or approving digital health products for technology vendors on the basis of their efficacy, quality, or impact on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process?** CMS has an opportunity to establish or facilitate a **marketplace for digital health solutions** for vendors, particularly focusing on tools that directly impact patient outcomes and care coordination for high-need populations. This marketplace could help providers and payers identify vetted solutions. Criteria for inclusion in such a marketplace might emphasize adherence to proposed standards like **Public-Benefit FHIR Endpoint** and support for **TEFCA Community-Service Endpoints**, ensuring data accessibility. It could also consider a tool's capacity for **AI-enabled Care Navigation**, with clear guardrails for human oversight and error rates, as well as its ability to enable access for **Authorized Representatives for Patient Access APIs**, promoting accessible care.

7. Value-Based Care Organizations

- **VBCO-1. What are the specific opportunities and challenges that health technology brings to improving patient access to care for value-based care organizations? Opportunities:** For VBC organizations, technology offers the ability to precisely identify, engage, and manage high-cost, high-need patients. Our **AI-enabled Care Navigation Copilot** helps extend high-touch support at scale, improving patient engagement and adherence. The integration of non-medical and public benefit data (via **Public-Benefit FHIR Endpoint** and **TEFCA Community-Service Endpoints**) allows VBCs to proactively address root causes of poor health, rather than reactively managing acute episodes, thereby reducing costs. **Challenges:** The primary challenge is the lack of comprehensive, interoperable data across the clinical, non-medical, and public benefit sectors. Without this data, VBC organizations struggle to accurately stratify risk, coordinate truly holistic care, and measure the ROI of non-medical interventions.

- **VBCO-3. What specific features and functionalities of health technology are critical to successful value-based care organization adoption?** For VBC organizations, critical functionalities include robust population health management tools, sophisticated care coordination capabilities, and deep analytics. These functionalities are critically dependent on access to comprehensive data, which necessitates:

    - **Public-Benefit FHIR Endpoint:** To understand a patient's full risk profile tied to non-medical interventions and integrate this into care planning.
    - **TEFCA Community-Service Endpoints:** To track and attribute community-based interventions to health outcomes, demonstrating value for upstream investments.

- **AI-enabled Care Navigation Copilots:** To scale high-frequency, administrative support that drives adherence and reduces avoidable utilization, extending human capacity.
- **Authorized Representatives for Patient Access APIs:** To ensure data accessibility for care teams managing complex patients who may face digital barriers.

- **VBCO-5. What role, if any, should CMS have in reviewing or approving digital health products for value-based care organizations on the basis of their efficacy, quality, or impact on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process?** CMS's role in reviewing VBC digital health products can be minimal if robust, outcome-based incentives are in place. By defining desired patient outcomes and cost targets, CMS empowers VBC organizations to select innovative digital tools that best achieve these results. This fosters a competitive market where VBCs naturally seek solutions supporting desired outcomes, including driving forward non-medical and medical integration. By primarily measuring overall VBC performance, CMS allows the market to drive tech effectiveness.

- **VBCO-6. What types of resources should CMS provide to support value-based care organizations in the adoption and use of digital health products (for example, training, technical assistance, guidelines)?** CMS should evolve payment models to fully support and incentivize the use of technology for comprehensive care management, including non-medical care, by **moving away from time-based incentives towards value-based care incentives.** This evolution should include:

- **Incentivizing the adoption of standards** for exchanging public benefits and community service data, building on **Public Benefits FHIR Endpoint** and **TEFCA Community-Service Endpoints**.
- **Reimbursing for AI-enabled care navigation**, recognizing its efficiency and impact on patient outcomes, as detailed in the **AI-enabled Care Navigation Copilot and Patient Support** proposal.
- Crucially, **updating the CMS-HCC model to include non-medical Z-codes** will provide the financial justification for VBC organizations to invest in the comprehensive, tech-enabled non-medical interventions necessary to improve outcomes for high-need Medicare and Medicaid beneficiaries, aligning payment with true complexity. Additionally, CMS can provide valuable resources by offering **evaluation support and sharing successful case studies** of VBC organizations effectively leveraging digital health products and integrating non-medical care. This knowledge sharing would enable other organizations to learn from proven models and accelerate adoption.

Conclusion:

Pair Team believes that the next frontier in improving Medicare and Medicaid beneficiary health and empowerment lies in intelligently integrating non-medical care with clinical care, enabled by a forward-looking technology ecosystem. Our specific proposals for **Public-Benefit FHIR Endpoint**, **TEFCA Community-Service Endpoints**, **Authorized Representatives for Patient Access APIs**, an **AI-enabled Care Navigation Copilot and Patient Support**, and **Z-code Inclusion in Risk Adjustment** directly address critical gaps in data, access, and payment alignment, which are essential for unlocking the full potential of digital health for high-need populations across both programs.

By implementing these recommendations, CMS can facilitate seamless data flow, reduce administrative burden, incentivize upstream investments in non-medical

support, and ultimately empower Medicare and Medicaid beneficiaries with the comprehensive support they need to thrive.

Thank you for your consideration of our feedback.

Sincerely,

Neil Batlivala